

TREATMENT PLAN

CLIENT	CASE ID	PLAN DATE
Deepak S.	DS-001 (SLEEP-004)	2026-10-01
AGE	OCCUPATION	SUGGESTIBILITY
56	Principal Engineer (EE/CS)	76% Physical

PRESENTING CONCERNS

Disturbed sleep — sleeping longer (6-7 hours) but waking exhausted. Mind stays active during sleep ("like the engine is always idling"). Recently relocated internationally. Reports mental hyperactivity with business planning and digital media processing during sleep. Not anxiety-related.

CLINICAL ASSESSMENT

Suggestibility Profile:	76% Physical (highly responsive to direct, literal language)
VAK Primary:	Kinesthetic
Hypnotic Response:	Strong — arm-raising induction successful (all 4 nods), visible hand levitation, hand-to-forehead challenge held, rapid reactional deepener re-entry
Clinical Impression:	Disturbed sleep secondary to new-environment data overload. Brain processing novel stimuli (language, faces, culture, spatial maps) at shallow depth. Three sleep stages not completing due to cognitive backlog from digital media and business planning at bedtime.
Prognosis:	Good — motivated, responds well to Physical-lane language, strong hypnotic responsiveness

TREATMENT PROTOCOL

Modality:	Clinical hypnotherapy (vocational/avocational self-improvement)
Lane:	Physical suggestible (direct, literal, body-first)
Duration:	6-8 sessions (6 core + 2 contingency)
Frequency:	Every 1-2 weeks
Reassessment:	Session 4 (independence pivot decision point)

SESSION	FOCUS	KEY TECHNIQUES	TARGET OUTCOME
1	Assessment + Education + First Induction	Suggestibility test, Theory of Mind, Eagle arm-raise, progressive relaxation, Sorting Room, Depth Dial	Baseline established, first trance experience
2	THE INSTALLATION	10 post-hypnotic suggestions anchored in deep trance, ideomotor confirmation at each checkpoint	All 10 PHS installed and confirmed
3	Maintenance + Overactive Mind	Depth Anchor Chain, Processing Vault, Train Technique, Auto-Pilot mode	Night-waking ease, cognitive quieting
4	Independence Pivot	7-step self-hypnosis teach, Natural Pharmacy, Childhood Sleep Memory regression	Client runs ceremony independently
5	Fine-Tuning	Street Metaphor / body scan / simplified 3-step (based on data)	Effortless sleep onset
6	Graduation	Sleep Temple room-by-room tour, ceremony, turbo mode	All goals met, full independence
7-8	Contingency / Reinforcement	As needed based on progress data	Only if data requires

PROGRESS INDICATORS (SUBJECTIVE + CLINICAL)

MEASURE	BASELINE	TARGET	METHOD
Morning restedness	4/10 (exhausted)	7-8/10 (refreshed)	Client self-report (1-10 scale)
Sleep onset ease	Difficult (~35 min)	Natural, effortless	Subjective: Difficult → Effortless scale
Night-waking	Frequent, hard to return	Rare, returns easily	Client self-report
Daytime energy	Low, persistent fatigue	Consistent throughout day	Client self-report (1-10 scale)

Sleep ceremony compliance	Starting	Effortless nightly habit	Homework checklist
Self-hypnosis confidence	Not yet learned	8+/10 (fully independent)	Client self-report (post-Session 4)

Progress is measured through subjective experience and clinical observation. Personal sleep tracker data is for the client's own awareness — therapy outcomes are not determined by device readings.

CLIENT RESPONSIBILITIES

- Nightly Sleep Ceremony at 10:30 PM (4-7-8 breathing, tension-release, settling practice, screen cutoff)
- Dream journal entries reported at each session
- Attend all scheduled sessions (reschedule with 24-hour notice)
- Maintain consistent bedtime within 30-minute window
- No alcohol as sleep aid
- Report new symptoms, medication changes, or concerns promptly
- Listen to session recordings between sessions (when provided)

THERAPIST RESPONSIBILITIES

- Provide safe, structured hypnotherapy sessions within scope of practice
- Monitor progress against subjective indicators at each session
- Adjust techniques based on client response and suggestibility lane
- Refer to licensed mental health professional if issues outside scope arise
- Maintain HIPAA-compliant records and confidentiality
- Reassess treatment plan at Session 4 decision point

EVIDENCE BASE

- Cordi, M.J. et al. (2014). Deepening sleep by hypnotic suggestion. *Sleep*, 37(6), 1143-1152.
- Chamine, I. et al. (2018). Hypnosis intervention effects on sleep outcomes: a systematic review. *J Clin Sleep Med*, 14(2), 271-283.
- Lam, T.H. et al. (2015). Hypnotherapy for insomnia: a systematic review and meta-analysis. *Complement Ther Med*, 23(5), 719-732.

AI DISCLOSURE

This treatment plan was developed by the therapist with AI-assisted tools for documentation and session planning. All clinical decisions, therapeutic techniques, and client interactions are performed by the therapist. AI tools are used in an advisory capacity only — all AI-generated content is reviewed and approved by the therapist before use.

SB 577 Disclosure: Jithendran Sellamuthu, C.MH. is not a licensed physician, psychologist, or psychiatrist. Services are provided for vocational/avocational self-improvement under CA B&P Code §2908. This is not a substitute for medical or psychological treatment.

No Guarantee of Outcomes: Individual results vary based on suggestibility, compliance, and overall health. VRishi Hypnotherapy does not guarantee specific outcomes, cure rates, or timelines. Progress indicators are research-based targets, not promises.

Therapeutic Exclusions: Clients on anti-depressant, anti-psychotic, or anti-anxiety medications, or with psychiatric history/suicidal ideation, must be referred to licensed mental health professionals.

Confidentiality: All information kept confidential per HIPAA Privacy Rule (45 CFR §§160, 164) and CA law, except where disclosure is required by law.

Client Signature / Date

Therapist: Jithendran Sellamuthu, C.MH. / Date